SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Continuous fluorouracil + radiotherapy

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Locally advanced rectal cancer · NCCP 00421 · version 6 · 7-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

**Schedule**
Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

### Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity

### Pelvic radiotherapy
LOCAL RADIATION TREATMENT

- ☐ Bowel irritation / diarrhoea
- ☐ Pelvic skin soreness
- ☐ Urinary symptoms
- ☐ Fatigue
- ☐ Possible lasting bowel / bladder effects
- ☐ Sexual function / fertility effects

Confirm treatment aim, expected benefit, reasonable alternatives and no-treatment/supportive-care options. Discuss DPD, infection, mouth/bowel/skin toxicity, cardiac symptoms, fertility, contraception, breastfeeding and interactions. Discuss separate radiotherapy consent and possible late bowel, bladder, sexual and fertility effects. Explain line/pump risks and response to malfunction.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00421 v6.

SACTCheck v0.84.32
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